

Mental Health Resources

- There are resources for every situation and every level of stress. There are many helpful resources available at UTSC's Health and Wellness Centre <https://www.utsc.utoronto.ca/hwc/>.
- An important part of the University experience is learning how and when to ask for help. Please take the time to inform yourself of available resources.
- Community Crisis Response Coordinator: Khadija Uddin: k.uddin@utoronto.ca
- Campus Police: (416) 978-2222
- Off-Campus: Good2Talk - a post-secondary (24/7) helpline (1-866-925-5454).

Mental Health Resources

- If you or someone you know is in crisis:
 - If you're in immediate danger or need urgent medical support, call 9-1-1.
 - If you or someone you know is thinking about suicide, call or text 9-8-8. Support is available 24 hours a day, 7 days a week.



Psychological Disorders 1

Introduction to Psychological Disorders & Major Classification of Psychological Disorders:

Anxiety Disorders

Lecture 16

Summer 2026

Dr. Anna Michelle McPhee

- By the end of class, students should be able to...
 - Define **psychological disorders** and **abnormal psychology**.
 - Describe the process of diagnosing psychological disorders using the DSM-5.
 - Describe the central features of **anxiety disorders**, and describe the main differences between **generalized anxiety disorder**, **phobic disorders**, and **panic disorder**.
 - Contrast **specific phobias** with **social phobia**, and comment on how **preparedness theory** might apply to phobic disorders.

Learning Objectives

- **Psychological disorder:** “a syndrome characterized by clinically significant disturbance in an individual’s cognition, emotion regulation or behaviour that reflects a dysfunction in the psychological, biological, or development processes underlying mental functioning.”
 - Can be understood both categorically and as part of a continuum.
- What causes psychological disorders?
 - Causal factors include genetics, biological, cognitive, developmental and social aspects.

Figure 14.1 A Continuum from Normal to Psychological Disorder

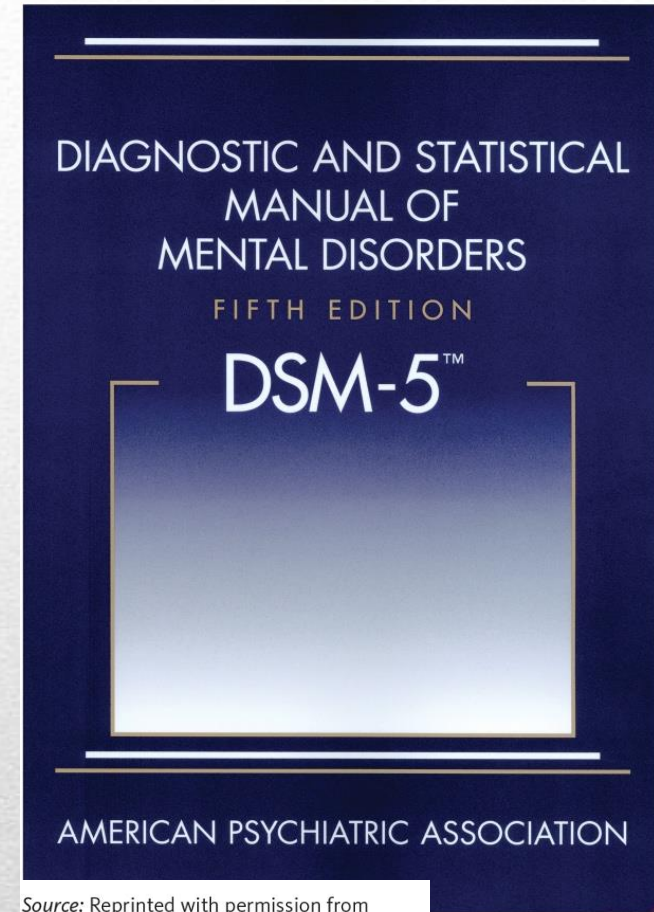
	NORMAL	MILD	MODERATE	PSYCHOLOGICAL DISORDER—LESS SEVERE	PSYCHOLOGICAL DISORDER—MORE SEVERE
Emotions	Good alertness and positive emotional state.	Feeling sad or down temporarily, but not for long.	Feeling sad, but a strong positive experience such as a good grade could lift mood.	Intense sadness most of the day with some trouble concentrating and some loss of appetite.	Extreme sadness all of the time with great trouble concentrating and complete loss of appetite.
Cognitions	"I'm not getting the grades I want this semester, but I'll keep trying to do my best."	"I'm struggling at school this semester. I wish I could study better, or I'll fail."	"These bad grades really hurt. This may set me back for a while. I'm really worried."	"I'm so worried about these grades that my stomach hurts. I don't know what to do."	"These bad grades just show what a failure I am at everything. There's no hope; I'm not doing anything today."
Behaviours	Going to classes and studying for the next round of tests. Talking to professors.	Going to classes with some trouble studying. Less contact with others.	Skiping a few classes and feeling somewhat unmotivated to study. Avoiding contact with professors and classmates.	Skiping most classes and unable to maintain eye contact with other people. Strong lack of motivation.	Unable to get out of bed, eat, or leave the house. Lack of energy and frequent crying.

What are Psychological Disorders?

- **Abnormal psychology:** the study of psychological disorders.
- **Onset:** the chronological age or situational period when the symptoms of a disorder first appear in an individual.
 - E.g., When does it (usually) start?
- **Prognosis:** the likely course (trajectory, development) of a disorder.
 - E.g., What will happen next? When will the disorder go into remission?
- **Risk Factors:** a set of biological, psychological and social characteristics that increase the likelihood of having the disorder.
- **Etiology:** the biological, psychological and/or social causes of a disorder.
 - E.g., What causes the disorder? What makes one individual more likely to have the disorder than another individual?
- **Comorbidities:** other psychological or physical disorders that frequently co-occur with the disorder in question.
 - E.g., What other disorders often appear with this one?

Abnormal Psychology- Key Terms

- **Psychological Disorders:** Patterns of deviant and dysfunctional behaviours, thoughts and/or feelings that cause significant distress, and may even be dangerous, and last for a specific amount of time.
- Most DSM disorders have 3 diagnostic criteria in common:
 1. Causes significant distress/affects functioning.
 2. Cannot be attributed to substance use or other medical condition.
 3. Cannot be better described by another DSM diagnosis.



Source: Reprinted with permission from the DSM-5.

Diagnosing Psychological Disorders

- To illustrate the DSM components, we'll start by talking about one common type of mental illness:
 - **Anxiety Disorders:** a group of disorders marked by excessive fear and anxiety.

Fear



Anxiety



Anxiety Disorders

- Fear and anxiety are adaptive reactions to threats.
- It is typical to be fearful of a hungry lion.
- Or anxious about an upcoming exam.
- Why?
 - Remember Maslow's needs hierarchy!



Anxiety Disorders

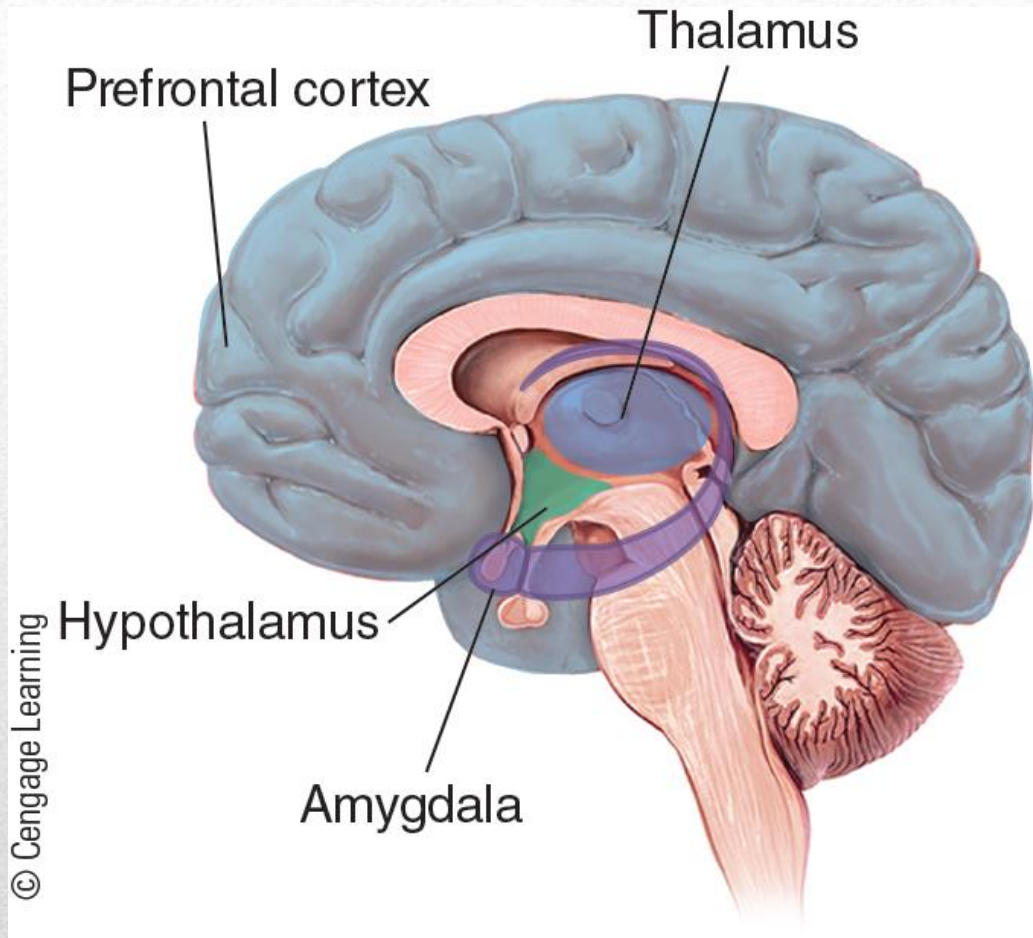
- However, anxiety that interferes with normal functioning is maladaptive.
 - Decreases our fitness for survival.
- This **pathological anxiety** can be classified as one of many **anxiety disorders**.
- They are very common (an estimated 1 in 4 adults in Canada will experience one).
- Prevalence of anxiety disorders varies across gender and ethnicity.

	UBC Domestic Undergraduate	UBC International Undergraduate
Diagnosed or treated by a professional for a mental health issues	13%	16%
So depressed it is difficult to function	34%	47%
Overwhelming anxiety	53%	52%
Felt things were hopeless	52%	57%

Anxiety Disorders

- The DSM-5 recognizes 12 types of anxiety disorders.
- We will examine:
 - **Generalized Anxiety Disorder** (most common)
 - **Phobic Disorders**
 - **Panic Disorder**

Anxiety Disorders



- **Generalized Anxiety Disorder** (GAD): an anxiety disorder in which worries are not focused on any specific threat.
- What is the **etiology** of the disorder?
- What **comorbidities** are there?
- We can use GAD as a case study for examining the different parts of the DSM.

Figure 14.17 Brain Circuits and GAD

Generalized Anxiety Disorder

- Diagnostic Criteria:

1. Excessive anxiety and worry, occurring more days than not for at least 6 months, about more than one event/stressor.
2. The individual finds it difficult to control the worry.
3. Three or more of these symptoms:
 - a) Restlessness
 - b) Fatigue
 - c) Concentration deficiency
 - d) Irritability
 - e) Muscle tension
 - f) Sleep disturbance

Generalized Anxiety Disorder: Criteria

- Diagnostic Criteria (Three Things Common to all Psychological Disorders):
 4. Causes significant distress/affects functioning.
 5. Cannot be attributed to substance use or other medical condition.
 6. Cannot be better described by another DSM diagnosis.

Generalized Anxiety Disorder: Criteria

- **Onset** of GAD rarely occurs prior to adolescence.
- Median age for diagnosis is age 30.
 - But many patients report having anxiety symptoms for a long time before reporting them.
- In the population, the level of anxiety is constant throughout the lifespan.
 - Content of worries change.

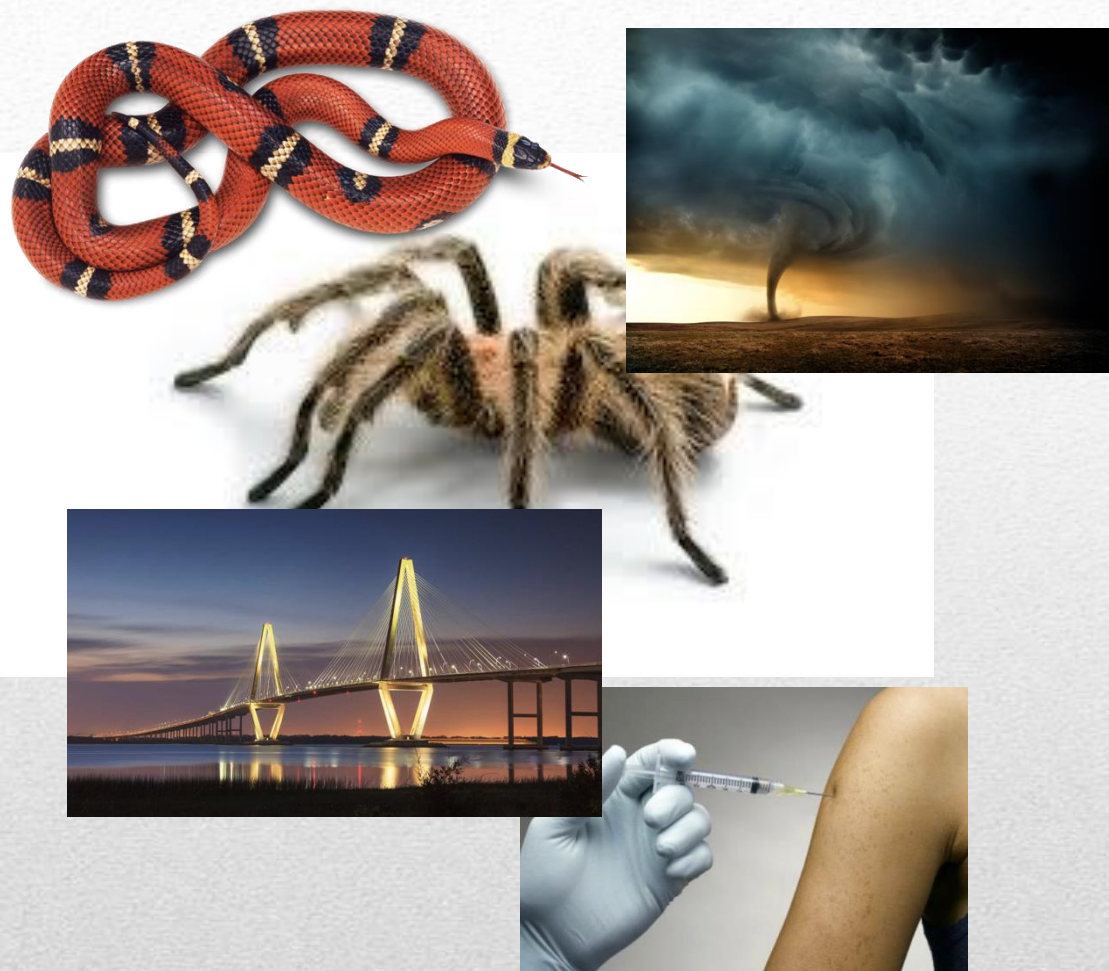


Generalized Anxiety Disorder: Onset

- For individuals, severity of symptoms waxes and wanes across the lifespan.
- **Prognosis:** Full **remission** is rare.



Generalized Anxiety Disorder: Prognosis



- A more specific type of anxiety disorder is a **phobic disorder**.
 - Disorder characterized by marked, persistent, excessive fear of specific objects, activities, or situations.
 - Usually the person recognizes the irrationality of their fear...
 - ...but cannot control it.

Phobic Disorders

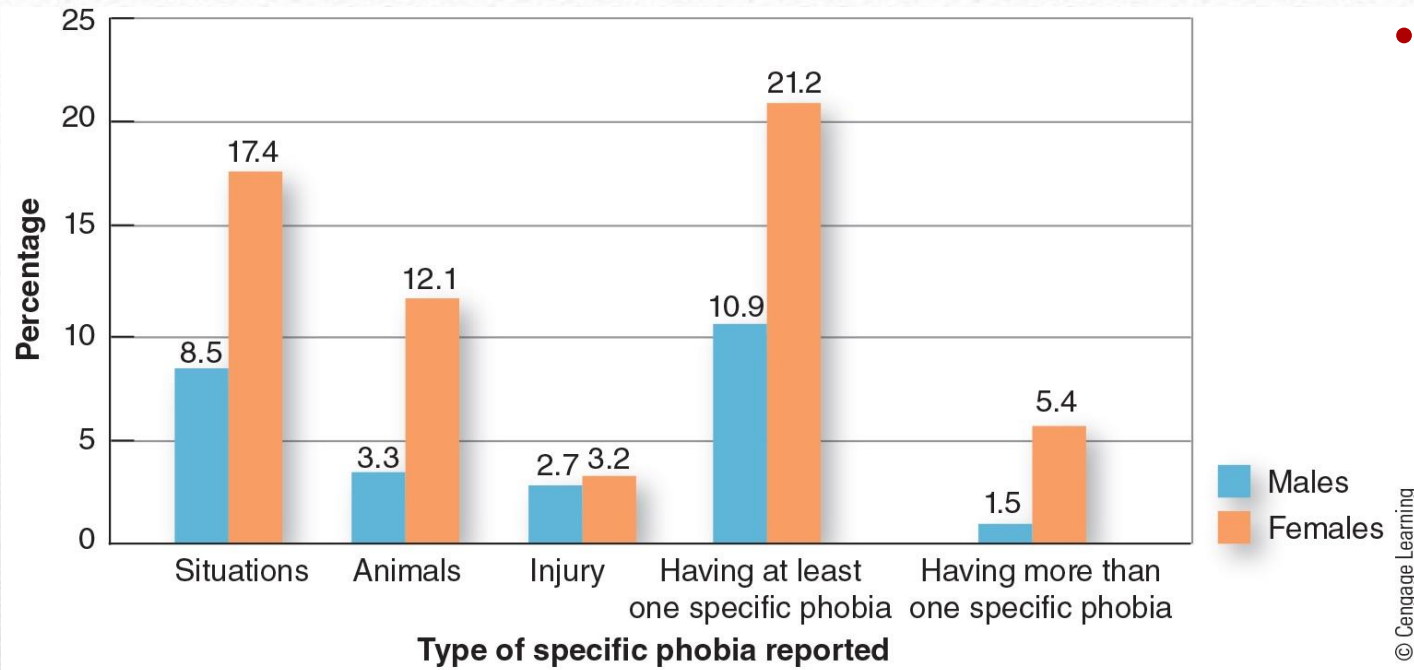


Figure 14.16 Prevalence of Different Types of Specific Phobias

Phobic Disorders

- **Specific phobia** examples:
 - Animals (e.g., dogs, cats, spiders, snakes)
 - Natural environments (e.g., earthquakes, darkness)
 - Situations (e.g., elevators, enclosed spaces)
 - Medical events (e.g., blood, injections, injury)
 - Other (e.g., loud noises, costumed characters, choking)
- **Social phobia**—maladaptive fear of being publicly humiliated or embarrassed (13% prevalence).

- Why are phobic disorders so common?
- **Preparedness Theory**
 - We may be evolutionarily adapted to fear certain types of stimulus.
 - Evidence for this hypothesis comes from **conditioning**.
 - Monkeys can easily be conditioned to fear snakes, but not flowers.
- These fears may be overdeveloped in some individuals.

Phobic Disorders

- A final type of anxiety disorder is **panic disorder**.
- Feelings of **panic** are normal when faced with immediate, life-threatening danger.
- But many individuals experience panic even when not in danger.
- **Panic disorder**—sudden occurrence of multiple psychological and physical symptoms typically associated with terror.
 - Shortness of breath
 - Heart palpitations
 - Sweating
 - Dizziness
 - Derealisation (feeling that the world is unreal)
 - Fear of death/“losing one’s mind”

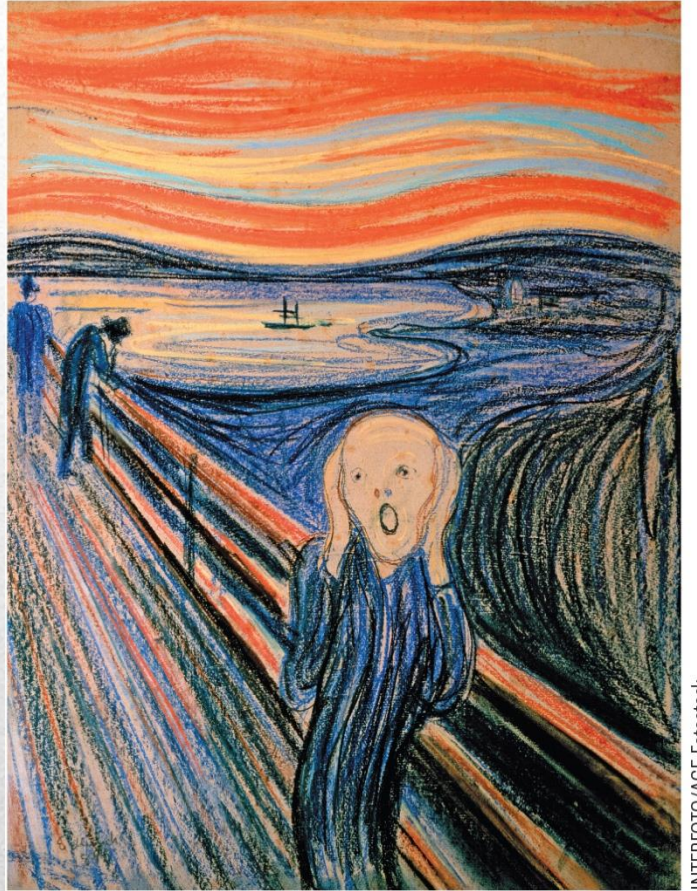
Panic Disorder



Panic episodes are fairly common.

Panic Disorder

- **Panic episodes** are relatively common.
- About 1/3 of Canadians experience a panic attack once or more per year.
- Typically during extreme stress.
- These occasional panic episodes are not sufficient for a diagnosis.
- To be diagnosed, an individual must experience:
 - Recurrent, unexpected attacks.
 - Significant fear of another attack.



INTERFOTO/AGE Fotostock

Panic Disorder & Agoraphobia

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